

FHIA-06 Bringing Healthcare Commerce into the Light

Transforming Data into Information, Knowledge, and Insight

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AUGUST ??, 2026

FUTURE HEALTHCARE INFORMATION ARCHITECTURE (FHIA) SERIES

Healthcare decisions are often made with remarkably limited information about alternatives, costs, experience, and outcomes. This paper examines how a national longitudinal patient history could bring healthcare commerce into the light—giving patients, providers, payers, and government better information while enabling informed choice and competition to improve access, effectiveness, and value.

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Introduction

Recently, I developed severe neck pain that made it difficult to look left, right, or up. I called my doctor's office and was told they could get me an appointment with a spine specialist—in about a month. That is a long time to endure pain and limit normal activities. Were there other specialists who could see me sooner? How would I find them? And how would I know whether they were good choices?

A friend needed to reschedule a video appointment with a nurse practitioner because of another important medical appointment. The next available video appointment was almost two months later. Was another provider available sooner? There was no easy way to know.

I read an account on Facebook from a woman with a lump on her chest that was growing by the hour and becoming hot. One emergency room prescribed a low-dose antibiotic, heat, and a mammogram. When she continued to worsen, she went to another ER that performed blood tests and an ultrasound and high-dose antibiotics intravenously. She was told she had severe cellulitis that could have progressed to deadly sepsis.

These are anecdotes and cannot establish whether the care provided was clinically appropriate. But they illustrate a larger problem: **healthcare consumers often make important decisions with remarkably little information about their alternatives.** Who is available? What will it cost? What experience does the provider have? What results have other patients experienced?

Even when a physician recommends a test or treatment, a payer may require prior authorization, additional evidence, or that a less expensive treatment be tried first.

Imagine buying a car this way. You may not know its ultimate cost before committing to buy it, have little information about its reliability, and have someone else paying most of the bill determine which options you can purchase. We would hardly expect such a marketplace to produce vigorous competition based on price and performance.



Healthcare isn't car buying. Medical decisions require specialized knowledge, emergencies can eliminate choice, and outcomes vary among patients. But those differences don't explain why information about healthcare choices should remain so difficult to obtain.

Bringing healthcare commerce into the light means giving consumers better information about availability, cost, experience, and observed outcomes. Freedom of choice becomes far more powerful when people have the information necessary to exercise it.

Healthcare Is a Market Without Normal Market Information

In most markets, consumers expect information before making a purchase. They can compare products, prices, availability, features, warranties, and other buyers' experiences. Competition gives suppliers an incentive to offer better products, better service, or lower prices.

Healthcare operates differently. Patients may be able to choose among physicians, hospitals, imaging centers, laboratories, insurance plans, and other healthcare organizations, but much of the information needed to make those choices is difficult or impossible to obtain.

Consumers have attempted to fill the information vacuum themselves. They ask friends and neighbors, post questions to community Facebook groups, and consult online provider ratings. These sources can be useful for learning whether a physician communicates well, the staff is helpful, or appointments run on time. But a five-star review tells us remarkably little about what



may matter most: How much experience does this provider have treating patients like me? What treatment was provided? What did it cost? And what happened to those patients afterward?

Even a review saying, "Dr. Jones fixed my shoulder," provides limited evidence. Was that patient's shoulder problem the same as mine? Were we similar in age and health? Had we tried the same previous treatments? Would the patient have improved regardless? Patient reviews can describe an experience, but they rarely provide enough information to compare healthcare effectiveness.

Price can be equally difficult to understand. The amount billed may bear little relationship to the amount accepted from a payer, and what the patient ultimately pays depends on negotiated rates, deductibles, copayments, coinsurance, network status, and the services that follow. The least expensive initial treatment may not be the least expensive course of care if it fails and additional treatment becomes necessary.

The opacity extends beyond providers. Consumers also choose payers, either directly or from alternatives offered by an employer or government program. They can compare premiums, deductibles, copayments, and provider networks, but it can be much harder to understand how the payer may constrain future healthcare choices.

What diagnostic procedures require prior authorization? What treatments must be tried before another will be approved? How often are authorization requests denied? How long does approval typically take? How restrictive is the provider network when specialized care is needed? These differences may be relatively unimportant when selecting an insurance plan but become extremely important after someone becomes ill.



Healthcare transparency therefore needs to mean more than publishing prices. A transparent healthcare marketplace would make the information that materially affects a healthcare decision visible **before the decision must be made**.

Today, without that transparency, patients may choose providers based on referrals, recommendations from friends and neighbors, online reviews, reputation, location, what insurance they accept, or simply who can give them an appointment. Those sources can be useful, but they provide little objective information about the provider's experience, cost, or results with patients like them.

What patients need when choosing a provider is better information about:

- **Availability:** Who can provide the care, where, and how soon?
- **Experience:** How much experience does the provider have treating patients with similar conditions and circumstances?
- **Cost:** What is the likely cost to the patient and payer—not just for the initial service, but for the course of care?
- **Observed outcomes:** What happened afterward to comparable patients receiving similar care?

The same problem exists when choosing a payer. Today, consumers commonly compare premiums, deductibles, copayments, provider networks, and covered benefits. What may be much less apparent is how the payer will affect their choices after they become ill and need care.

What consumers need when choosing a payer is better information about:

- **Provider access:** How broad is the network, particularly for specialists, and how readily can members obtain needed care?
- **Prior authorization:** Which tests, treatments, and medications require approval before they can be provided?
- **Restrictions:** What treatments must be tried first, and what other requirements can limit the care recommended by a physician?
- **Approval and denial experience:** How often are requested services approved or denied, and how long do those decisions typically take?
- **Appeals:** How frequently are denials reversed, and how long does the appeal process take?

None of this information can guarantee a successful outcome or identify a single "best" provider or payer. Healthcare is too complex, and patients and their circumstances are too different, for such certainty. But uncertainty does not make information worthless.

A functioning market does not require perfect information. It requires enough information for buyers to make meaningful choices and for suppliers to compete for those choices. Healthcare frequently provides neither.

What Happens When Healthcare Has to Compete?

We do not have to rely entirely on theory to consider what greater consumer choice might do to healthcare. Some healthcare services operate much more like conventional markets because consumers pay directly and can choose among competing providers.

LASIK eye surgery provides an interesting example. Because LASIK is generally elective, non-urgent, and paid directly by the consumer rather than a third-party payer, prospective patients have both the opportunity and financial incentive to compare providers. They can ask what the procedure will cost, investigate alternatives, and decide whether and where to have it performed.

Research presented to Congress found that competition helped keep LASIK prices down. The inflation-adjusted price of conventional LASIK declined nearly 30 percent over roughly a decade. Researchers attributed the competitive environment partly to consumers' ability to obtain price information relatively easily and to ophthalmologists' ability to enter the market and compete for patients. Other government analysis subsequently reported a roughly 25 percent inflation-adjusted price decline from 1999 through 2011 while quality improved.

LASIK also demonstrates that competition alone does not solve the information problem. Researchers found that providers did not always package services consistently, making direct price comparisons difficult. Quality varied among providers, and consumers continued to rely heavily on word-of-mouth recommendations. Some advertised prices could also be misleading.

That distinction is important. The lesson from LASIK is not that every healthcare service should operate like an elective self-pay procedure. Emergencies cannot be comparison-shopped, many medical decisions require substantial clinical expertise, and insurance protects patients from costs they could not reasonably bear themselves.

The more useful lesson is that **when consumers have meaningful choices, useful information, and an economic reason to consider the alternatives, providers have an incentive to compete for their business.**

That competition does not have to be limited to price. Providers can compete on availability, convenience, experience, service, technology, and observed outcomes. A provider who can deliver comparable care sooner, at lower cost, or with better observed results has something meaningful to offer prospective patients—if patients can see the difference.

Much healthcare suppresses those signals. The patient may not know the price, the payer may determine which providers or treatments are available, and information about provider experience and outcomes may be unavailable. When consumers cannot distinguish among alternatives, providers have less reason to compete on differences consumers cannot see.



Bringing healthcare commerce into the light would give consumers the information and freedom of choice needed to make competition meaningful. Providers who deliver better outcomes, greater access, or better value would have an opportunity to attract more patients. Those who do not would have an economic incentive to improve. That is how informed choice can turn transparency into a force for improving healthcare.

Payers Are Making Decisions in the Dark Too

During a business discovery discussion years ago, an actuary made a comment that stayed with me: **“There are no bad policies, only bad rates.”** His point was that insurance

ultimately depends on collecting enough money across a population to pay the claims that population will generate.

Insurance companies are not charities. They pool risk so that an individual facing a major illness does not have to bear the entire cost alone. The money to pay healthcare claims must ultimately come from premiums, employers, government programs, or patients themselves. Insurers must collect enough to pay those claims and their operating expenses and, for investor-owned companies, earn a return sufficient to attract capital.

That gives payers a legitimate reason to question whether healthcare expenditures are necessary and appropriate. Paying every claim and approving every requested treatment without scrutiny would eventually increase insurance costs for everyone.

The problem is that payers may be making those decisions with limited information.

Consider a request for an expensive MRI. Ideally, the decision would consider the patient's symptoms, clinical history, previous injuries and illnesses, prior imaging, treatments already attempted, changes in the condition, and the physician's reason for ordering the test. Without ready access to that broader clinical context, a payer may have little choice but to rely on the documentation submitted with the request and rules developed for patient populations.

Those rules can become blunt instruments:

- Try physical therapy first.
- Obtain an X-ray before an MRI.
- Try the less expensive medication first.
- Provide additional documentation before approval.

Such requirements may be reasonable for many patients. But the treatment that usually makes sense for a population may not make sense for a particular individual. A requirement intended to avoid an unnecessary expense can delay diagnosis or treatment—and in some cases ultimately increase the total cost of care.

The same longitudinal information could strengthen fraud and improper-payment detection by allowing claims to be evaluated against a broader record of clinical events, diagnoses, procedures, and treatments rather than primarily against other claims. A suspicious claim could be examined in the context of what actually happened to the patient, making it easier to distinguish legitimate care from billing that warrants further investigation.

But the value of longitudinal information should not end when the claim is paid. It can also help answer the question that claims data alone rarely answer: **What happened to the patient afterward?** Did the patient improve? Was additional treatment required? Were there complications? And what was the total cost of care over time?

Those answers could provide evidence for improving future authorization policies. If an expensive treatment consistently produces good outcomes for patients with particular characteristics, that history should matter when a similar patient needs the same treatment. Conversely, if a less expensive treatment that is routinely required first frequently fails for certain patients and merely delays the treatment they ultimately receive, that should matter too.

Better information does not eliminate the payer's responsibility to control costs. It provides better tools to do so. The objective should not be to approve more care or deny more care. It should be to approve appropriate care faster, detect inappropriate expenditures more accurately, learn from previous costs and outcomes, and make future decisions based on the likely total course of care rather than simply the cost of the next claim.

What Should the Federal Government Do?

The federal government faces many of the same information problems as patients, providers, and payers—but on a much larger scale. **In 2026 alone, the federal government is expected to spend roughly \$1.9 trillion on Medicare, Medicaid, and other major healthcare programs.** It also influences the supply of healthcare professionals and sets rules that affect healthcare organizations nationwide.

With that scale comes responsibility for controlling costs, protecting public funds from fraud, and ensuring that federal programs work as intended. **Without sufficiently detailed and timely information, however, government actions can become blunt instruments.**

Targeting Fraud

Recent hospice fraud investigations in California illustrate both the scale of the problem and the difficulty of responding after fraud has become widespread. Federal authorities



suspended Medicare payments to **447 hospice providers and 23 home health agencies in the Los Angeles area**, involving an estimated **\$600 million in suspected fraud**. Investigations have found schemes involving patients who were not terminally ill, people enrolled in hospice without their knowledge, services that were never provided, and the misuse of identities. The concentration itself was extraordinary: state records showed more than half of California's hospice providers located in Los Angeles County, and investigators found dozens of hospice businesses associated with individual office locations.



447

Hospice providers suspended

\$600 MILLION

Suspected fraud

This is exactly the kind of problem that better national information should expose much earlier.

Instead of discovering hundreds of questionable providers after enormous amounts have been

billed, analytics combining enrollment, claims, patient histories, provider locations, and clinical evidence could identify implausible patterns as they develop and focus investigation on the providers creating them.

Concern about fraud has also led the federal government to freeze funding while requiring additional documentation and review. In January 2026, it froze access to certain child-care and family-assistance funding for five states. The states sued, and a federal court blocked the freeze while litigation proceeded. **Broad actions intended to stop fraud can entangle legitimate services and funding while adding litigation costs.**

Responding to Workforce Needs

Healthcare workforce policy provides another example of the long-term consequences of broad intervention. In the 1990s, policymakers were being warned of a coming physician surplus. The Balanced Budget Act of 1997 generally limited the number of residency positions for which individual teaching hospitals could receive Medicare graduate medical education funding, using fiscal year 1996 training levels as the baseline. Nearly three decades later, an aging population is consuming more healthcare, and many communities report difficulty obtaining timely access to physicians and specialists.

Wondering why you may need to wait several months to see a specialist?

The lesson is not that government should withdraw from healthcare. Government has responsibilities that individual patients, providers, and payers cannot perform. The question is **where federal involvement provides the greatest value and where decisions are better left to patients, healthcare professionals, payers, markets, and states.**

Facilitating Interstate Commerce

The Constitution provides a useful starting point. Article I gives Congress the power **“To regulate Commerce with foreign Nations, and among the several States.”** In *Gibbons v. Ogden* in 1824, Chief Justice John Marshall described commerce among the states as commerce **“which concerns more states than one.”**

Modern healthcare clearly crosses state boundaries. A patient may live in one state, receive treatment in another, and use insurers, pharmacies, laboratories, and other organizations operating nationally. During a lifetime, that patient may move repeatedly while decades of healthcare information remain scattered among organizations in multiple states.

A national longitudinal patient history does not require the federal government to decide which physician a patient should see, what a physician should prescribe, which treatment a payer should approve, or how much every provider should be paid. Those decisions can remain with the participants responsible for making them.

The federal government can establish the infrastructure and standards that allow trustworthy healthcare information to follow patients across state and organizational boundaries, with safeguards for privacy, security, auditability, and appropriate use.

Better national information could also make government oversight more precise. Claims combined with relevant clinical information could identify implausible billing and enable earlier investigations. Information about provider supply and demand could similarly reveal developing shortages before broad corrective policies become necessary.

The Interstate Highway System Facilitates Interstate Commerce



**The government built the infrastructure.
It doesn't drive the trucks or cars.**

The objective is not to replace state authority, private healthcare organizations, insurance companies, or individual choice with a national healthcare system. It is to provide infrastructure for a healthcare marketplace that already operates across state boundaries.

The federal role should be to bring healthcare commerce into the light—not to make every healthcare decision. National infrastructure can make information visible and trustworthy. Patients, providers, payers, states, and markets can then use that information to make better decisions.

Conclusion

Healthcare does not lack information. It lacks the ability to reliably bring together the right information about a patient, resolve inconsistencies, protect it, and make it available when and where it can improve a healthcare decision.

A national longitudinal patient history could assemble information now scattered across providers, hospitals, laboratories, imaging centers, pharmacies, payers, and other healthcare organizations. Common rules could determine who may see what information and for what purpose, while national access monitoring could record who accessed it, when, and why—protections that are difficult to achieve consistently while the history remains fragmented among thousands of independent systems.

The value, however, comes from what we can learn and do with that information. **Patients** could make better-informed choices based on availability, experience, cost, and observed outcomes. **Payers** could evaluate requests with greater clinical context, detect inappropriate expenditures, and learn whether expensive treatments ultimately produced better outcomes and lower total costs. **Providers** could spend less time reconstructing patient histories and have relevant information available before an encounter, while drawing on nationwide evidence about what treatments have worked for patients with similar circumstances.

None of this requires replacing today's healthcare systems or transferring healthcare decisions to the federal government. It requires an information infrastructure that connects what those systems already know, protects how that knowledge is used, and makes the results visible to those responsible for making decisions.

Bringing healthcare commerce into the light means giving patients, providers, and payers better information and greater opportunity to learn from what happened before. Better information enables better choices. Better choices create pressure for better care, better outcomes, and better value.

Acknowledgements

Microsoft Word was used for document preparation and spell checking. Grammarly was used to suggest improvements in clarity and wording. ChatGPT was used as a research and development aid to investigate topics, challenge assumptions, explore alternative approaches, refine and summarize text, and suggest concepts for illustrations. The author is responsible for the architecture, conclusions, content selection, and final editing.